

CITATION: Bader v. Evans, 2012 ONSC 5781

COURT FILE NO. 06-CV-318795

DATE: 20121011

**ONTARIO
SUPERIOR COURT OF JUSTICE**

BETWEEN: BILJANA BEADER (Plaintiff)

AND: CORY LLOYD EVANS (Defendants)

BEFORE: M.A. SANDERSON J.

COUNSEL: *Esther M. Cantor* for the Plaintiff
Blair Y. Nitchke for the Defendant

R U L I N G
re Threshold Motion

[1] This trial commenced on Tuesday, May 22, 2012 and continued until June 12, 2012. Only damages were in issue. On the last day of the trial, I heard a "threshold motion"/submissions from counsel for the Defendant to the effect that the Plaintiff had not satisfied the onus of proving that she fell under the statutory exception in s. 267(5) of the *Insurance Act*, i.e., in the accident of February 3, 2005, she did not suffer and does not suffer from a permanent serious impairment of an important physical, mental or psychological function.

[2] On the same day, the jury awarded the Plaintiff general damages of \$10,000, but nothing for past or future

- (a) income loss,
- (b) housekeeping expenses; or
- (c) treatment and rehabilitation expenses.

[3] After the jury returned, as did Brown J. in *Clark v. Zigrossi* 2010 ONSC 5403 (CanLII), I questioned whether counsel for the Defendant still wanted a formal ruling on the threshold issue. It appeared that by reason of the applicable statutory deductible, the Plaintiff would not recover any damages.

[4] In *Clark*, Brown J. wrote at paragraph 17:

[17] The threshold motion that has been brought before me is one that I understand is standard practice for defendants to bring. The context of this case leaves me scratching my head as to why I should have entertained a threshold motion at all. The jury's verdict of \$5,000.00 in general

damages reduced, as it must be, by the statutory deductible of \$15,000.00, results in an award of zero to the Plaintiff; that is to say, the Plaintiff did not obtain judgment. The threshold motion would result in exactly the same result - i.e. if granted, the action would be dismissed. I am alive to the fact that on the issue of costs, the Court of Appeal, in its decision in *Rider v. Dydyk* held that for the purpose of considering Rule 49 offers the deductions required to be made for an award of non-pecuniary loss under [section 267.5](#) of the [Insurance Act](#) should not be taken into account. So, I shall wait to see what Rule 49 offers, if any, there may have been. But that is the only possible relevance I see for a threshold motion in this case.

[5] In *Clark*, Brown J. wrote at paragraphs 8-11 and 18:

[8] A threshold motion may be brought before or at trial. Where the motion is brought during a jury trial, what role may the jury's verdict play in the judge's determination of a threshold motion?

That issue was nicely summarized by Reilly J., in *Parks v. Peter*:

The Ontario Court of Appeal has made it quite clear that the trial judge has an independent obligation to assess the threshold issue, which assessment and ruling might be in fact quite inconsistent with a jury verdict (resulting in at least one case "judge nullification" of a jury's award of damages). In *Kasap v. MacCallum*, [2001] O.J. No. 1719, the trial judge ruled against the plaintiff on the threshold issue while the jury deliberated on the issue of damages. The jury then returned and awarded the plaintiff substantial damages. In accordance with his ruling on the threshold issue the trial judge dismissed the action. Notwithstanding the jury verdict, the Ontario Court of Appeal upheld his ruling. The court stated at paragraphs 7 and 8:

The Legislature has left it to Judges to determine whether the threshold has been met. This will often overlap a jury's considerations; and particularly where the symptoms are subjective.

Nowhere does the legislature say that the Judge is bound to consider the jury verdict much less that the Judge is bound by any implied finding of credibility of the jury. By the same token the legislation does not suggest that a Trial Judge cannot, in the exercise of judicial discretion, consider the verdict of the jury. The legislation is clear: the Judge must decide the threshold motion, and in doing so, the Judge is not bound by the verdict of the jury. The timing of the hearing is in the discretion of the Trial Judge.

[9] In light of the differing roles of judge and jury, the timing of the hearing of a threshold motion during a jury trial takes on some significance. If the motion is heard and decided before the jury renders its verdict, then, one might argue, the jury has labored for naught. On the other hand, if the motion is heard and decided after the jury renders its verdict, might the judge be unduly influenced by the verdict? These are not new issues. On the threshold motion argued before me both counsel submitted large volumes of authorities. A review of them reveals two contending schools of thought.

[10] As to the possibility of the judge giving undue weight to the jury's verdict, in *Chrappa v. Ohm*, [1996] O.J. No. 1633 (Gen. Div.), Lax J. thought this was a real risk, which merited hearing and deciding the threshold motion before the jury rendered its verdict. ...

[11] In *Dinham v. Brejkaln*, [2005] O.J. No. 5409 (S.C.J.), Cumming J. took a different tack and voiced concern at para. 38 about how a decision by a judge on a threshold motion could undercut the work performed by the jury:

As an aside, I add that I question the utility of a regime having a judge mandated by statute to determine the so-called “threshold motion” when a six person jury has spent several days of their time as conscientious citizens hearing evidence, deliberating and arriving at a verdict late into the evening which in reality is of no effect because, unknown to the jury, a judge then makes a ruling contemporaneously which has the consequence that the entire exercise of the jurors is a waste of effort. ...

[18] The danger of threshold motions after the jury has rendered its verdict is, as has been indicated in Riley J.'s decision in *Parks v. Peter*, the possibility of inconsistent findings of fact by the trial judge and by the jury. If a jury has been selected as the trier of fact and if we are to preserve the jury system in civil cases in this Province, in my respectful view judges must take great care in avoiding interfering with findings of fact made by the jury which are implicit in their verdicts. Where the trial judge can infer what those findings of fact were, a jury verdict should not be interfered with, directly or indirectly, unless the rigorous test for setting aside a jury's verdict is met.

[6] I agree with Brown J. that insistence on a threshold ruling obviously creates the possibility of inconsistent findings.

[7] However, I note that in *Kasap v. Golden Champagne Enterprise*, 144 OAC 369, 2001 CarswellOnt 1554, where the trial judge heard and granted a threshold motion while the jury was deliberating, and a jury returned a verdict awarding substantial damages [suggesting it had found permanent, serious impairment of an important bodily function], the Ontario Court of Appeal, in response to counsel for the plaintiff/appellant's submission that the trial judge had been obliged to rule in conformity with the finding implicit in the jury's verdict, wrote at paragraphs 5-6:

[5] Kasap's credibility was central to both the determinations of the threshold issue and the jury's verdict as to the award of damages. It is conceded that if Kasap was believed when he testified that the accident caused him to suffer from fibromyalgia, then the threshold for bringing an action was met. The diagnosis of fibromyalgia is based in large measure on the subjective complaints of the individual afflicted. Kasap submits that it is the exclusive province of the jury to determine the facts. As a result, he submits that the trial judge was obliged to accept that Kasap was a credible witness and to rule that the threshold had been met.

[6] The question of whether a trial judge should rule on a threshold motion made at trial has been the subject of some considerable judicial comment and divergent views by trial judges. (For decisions that favour a ruling on the motion after a jury has made its determination see: *Kostiuk v. McIntyre* (1997), 1 C.C.L.I. (3d) 73; *Duguay v. O'Neill* 1996 CanLII 8227 (ON SC), (1996), 31 O.R. (3d) 22; and for decisions that favour a ruling before a jury has made its determination see: *Andre v. Roque* 2000 CanLII 22358 (ON SC), (2000), 48 O.R. (3d) 131 (Sup. Ct.); *Chrappa v. Ohm* (1996), 1 O.T.C. 248 (Gen. Div.); *Greer v. Horton* (1996), 18 O.T.C. 332 (Gen. Div.); *McMurray v. Davies*, [1996] O.J. No. 4834 (Gen. Div.)). None of the decisions at the trial level has, however, suggested that a trial judge is bound to follow the implicit finding of the jury. At its highest, the verdict of the jury is a factor for the trial judge to consider. See *Duguay v. O'Neill*, *supra*, at p. 23.

[8] The Ontario Court of Appeal held in *Kasap* that the judge may but is not required to consider the jury verdict, and is not bound by its findings, express or implied.

[9] Shortly after the threshold motion was argued, counsel requested that I defer deciding the threshold motion. They advised me in mid-July 2012 that they did require a formal ruling.

[10] I have struggled with the proper role of a trial judge in deciding such a motion. I am aware that in rare circumstances, s. 108(5) of the *Courts of Justice Act* appears to provide jurisdiction for a trial judge to pre-empt a jury's verdict.

[11] In my view, it is generally not my role, but the Court of Appeal's, to consider whether a jury's findings should be set aside. I would not have expressed my views here but for the insistence of counsel on a ruling. Given my respect for juries generally, I would have preferred to reach a conclusion consistent with the jury's.

[12] Having heard the same evidence as the jury over several weeks of trial, unfortunately I do not and cannot agree with their apparent conclusions on the seriousness of her impairment. In my view, the Plaintiff has met the onus of proving on a balance of probabilities that as a result of the accident of February 3, 2005, she suffered a serious, permanent impairment of an important physical and psychological function. She is continuing to suffer psychological and physical impairments and will likely continue to do so over the balance of her life. While she is capable of working part-time, she will be permanently disabled from working on a full-time basis. She continues to be clinically depressed as a direct result of the accident.

The Evidence

Overall

[13] The Plaintiff's case was based on the evidence of the Plaintiff, her family, her treating doctors, other treatment providers and experts, all to the effect that she has suffered and continues to suffer serious pain, headaches and psychological sequelae as a result of the accident. She gave evidence that she has always wanted to work full time at her profession. Because of her physical limitations from the accident, she retrained for less physically demanding work. She can do that less demanding work only part-time and with pain. She can do light, not heavy, housekeeping. She will need ongoing treatment.

[14] The Defendant's case was founded on the conclusions of Drs Cameron and Reznick that she was malingering, the accident provided no physical basis for her complaints and caused her no psychological impairment.

[15] I shall now review the evidence upon which I have concluded that the Plaintiff does suffer from a permanent impairment of important physical and psychological functions caused by the accident of February 3, 2005.

Plaintiff's Background

[16] The Plaintiff was born and raised in the former Yugoslavia. Prior to her arrival in Canada in 2000, she had obtained a Bachelor of Food Science degree in Belgrade in 1995 and had worked as a water quality analyst and nutritional advisor. At the time of the accident on February 3, 2005, the Plaintiff had not yet been able to obtain work in her field in Canada. She was seeking Canadian work that would allow her to utilize her training and skills. The Plaintiff gave evidence that her plan was eventually to find a job in her profession. That was not easily achieved because when she arrived in Canada she lacked English skills and Canadian work experience.

[17] Between immigrating to Canada with her husband and young son in 2000 and the accident on February 3, 2005, she had engaged in English training, and had given birth to a second son [in October 2001.] By February 2003 she was working two jobs: (1) cleaning at night for a janitorial service [approximately 16 hours a week] and (2) in a delicatessen in a Sobeys grocery store during the day [approximately 30 hours per week.]

[18] Both her pre-accident employers gave evidence. Although she was an excellent worker, her janitorial employer did not expect the Plaintiff to continue working for her over the long term because she seemed to have higher ambitions. Ms. McCabe, a representative of Sobeys, gave evidence that her job in the delicatessen department involved lifting up to 50 lbs, reaching, bending, and standing for prolonged periods and a fast physically demanding pace.

Pre-Accident Medical Issues

[19] The Plaintiff's pre-accident medical conditions included hyperthyroidism and low iron. Her hyperthyroidism, first diagnosed in 2003, was treated with medication. Her family doctor in 2003-2005, Dr. Tamari, gave evidence that when he first saw her in 2003, she was 35. She had not had a family physician during her first three years in Canada. In October 2003, she had been feeling fatigued. He prescribed thyroid medication and increased it over time. By the time of the accident, she was doing well. He also treated her low iron levels with medication.

[20] Dr. Reznick, the psychiatrist called by counsel for the Defendant, speculated, contrary to the Plaintiff's and her husband's evidence, that her pre-2000 experiences in Serbia may have caused psychological symptoms.

The Accident: February 3, 2005

[21] The Defendant admitted liability for the accident. He did not give evidence at trial.

The Circumstances of the Accident

[22] The Plaintiff was a front-seat passenger in a car driven by her husband. They were both wearing seat belts. He gave evidence that the Defendant's vehicle hit their car on the right front near the front

wheel [i.e., near where the Plaintiff was sitting.] The impact was "really strong." Their car was shaking. At the moment of impact, his wife's body moved to the left and her head hit his right shoulder.

[23] She gave evidence that upon impact she didn't think she had been seriously hurt. However, shortly thereafter, she started to feel pressure in her chest and to have difficulty breathing. She did not go to a hospital immediately.

[24] Within a few days, she started to have severe neck, upper back and shoulder pain, dizziness and headaches. She attended at the Emergency Department at St. Joseph's Hospital. She was frightened to ride in a car. She wasn't sleeping well. Nevertheless, her family needed her wages: She wanted to keep her job at Sobey's and continued to work during the day. Bending, a job requirement, increased her back pain and dizziness. When she came home she didn't do anything but rest. She didn't cook dinner or participate in other activities. Her husband filled in for her at night at the janitorial job.

[25] On March 26, 2005, while working at Sobeys, she fainted and was taken to hospital. She gave evidence that after that date, she could not physically continue to work. At that time, she felt "very bad." However, she hoped she would get better.

[26] She did not work again outside the home until March 2011. She maintained that she wanted to work but was incapable of doing so.

[27] Counsel for the Defendant submitted she was malingering. She submitted that her work at Sobeys was suggestive that she was not in as much pain as she contended.

[28] In a case where the primary defence was malingering, I thought it significant that she said that initially she did not think she had been seriously hurt. I accepted that she wanted to continue to work and did so as long as she could physically tolerate it. She only stopped working after she fainted at work.

Evidence of Treatment Providers

[29] Lois Lane, Dr. Steiner and Dr. Park were treating the Plaintiff at the time of the trial. Their evidence is dealt with here chronologically over various periods.

Dr. Tamari

[30] As noted earlier, Dr. Tamari, her first family doctor in Canada, had diagnosed hyperthyroidism in 2003 and had prescribed thyroid medication.

[31] He gave evidence that at the time of the accident, she seemed to be doing well on the medication. As of August 23, 2004, her TSH was normal.

[32] When he saw her just before the accident, on January 27, 2005, although she was feeling otherwise well, she had some pain in her thighs.

[33] After the accident of February 3, 2005, she was started on medication for anxiety, pain and headaches. His original diagnosis was "anxiety post-MVA."

Dr. Park: April 2005

[34] Dr. Park, her family doctor from April 2005 to the date of trial, gave evidence that when he first saw her on April 7, 2005, she was complaining she had been in an accident and was experiencing dizziness, anxiety, stiffness, chest pain, low back pain, vertigo pain, headache, left neck pain and numbness in her palms and fingertips. She was already on a number of pain medications and was taking physiotherapy. On examination, she had decreased range of motion of the neck, due to pain. She had tenderness in the trapezius.

[35] On April 13, 2005, Dr. Park filled out a disability certificate in relation to accident benefits, opining she could not return to work at that time. He mentioned neck strain with numbness in the palms, low back strain, headaches, dizziness, right shoulder pain, chest pain, anxiety and depression. On another part of the form, he opined that she had not had any similar pains or injuries prior to the accident. [In other words, in his opinion the injuries had been sustained as a result of the car accident.]

[36] When he saw her next on April 16, 2005, she was complaining of dizziness, headache, anxiousness, depression, neck pain, decrease in motion, tenderness. He diagnosed neck strain.

[37] He noted recurring complaints of dizziness.

Lois Lane: August 2005-January 2006

First Treatment Session

[38] Lois Lane ("Lane"), a social worker, started counselling the Plaintiff in August 2005 under the supervision of Dr. Steiner, a clinical psychologist.

[39] In 2005 and 2006 when Lane was treating the Plaintiff, Dr. Steiner met regularly with Lane, at least once a week, in a formal capacity, and 2-3 times a day informally. He sat in on sessions with the Plaintiff and Lane on three or four occasions.

[40] Lane gave evidence that she saw the Plaintiff once a week until January 2006 over 14 sessions. She was "struggling" and feeling guilty that her husband was working two jobs because she was unable to work.

[41] She said the Plaintiff was always in pain. She had difficulty sleeping. To the best of her knowledge, she had had no nightmares or sleep problems before the accident.

[42] As of January 2006 [the end of the first treatment session] although she wanted to work, in Lane's opinion she remained physically and mentally unable to do so.

Dr. Park: May 2005-August 2009

[43] Dr. Park gave evidence that she continued taking physiotherapy treatments throughout May, June and July 2005.

[44] In May of 2005, he prescribed iron pills.

[45] On June 29, 2005, he noted dizziness and headaches as well as numbness in her fingers. She was still taking physiotherapy. She continued to have tenderness in her neck muscles, neck and shoulder.

[46] Her condition was the same in August of 2005. As she was very depressed, he started her on anti-depressants.

[47] In November 2005, her pains were the same. She was still suffering from dizziness.

[48] Towards the end of 2005, Dr. Park referred her to a psychiatrist, Dr. Hannick, who increased the dosage of anti-depressants and prescribed sleep medication. Dr. Park referred to Dr. Hannick's note dated December 29, 2005: "confirming your diagnosis of major depression" and stating he thought she had post-traumatic stress disorder.

[49] Throughout 2006, Dr. Park noted that although her pain was slightly better, her depression was continuing. He referred her to a pain clinic.

[50] In January 2007 he noted she was continuing to suffer from pain and dizziness. He prescribed muscle relaxants. He made a diagnosis at that time of chronic pain.

[51] Nothing much changed between 2007 and 2009. She continued to complain of neck strain and depression, recurrent headaches, low back and neck pain. She complied with his recommendations. She continued to want physiotherapy and rehabilitation.

[52] In August of 2009, he noted depression and prescribed anti-depressant medication.

[53] Although Dr. Park said he did no specific validity testing to test her credibility, he said her complaints were consistent, i.e., she had the same pains in the same areas. There were periods when she had low iron and low TSH but they were corrected so they should not have been a continuing factor in her fatigue.

[54] In my view, Dr. Park was a credible witness. As her family physician he repeatedly treated the Plaintiff between 2005 and 2012.

Other Doctors who saw the Plaintiff in 2008

[55] Apart from Lane/the family physicians/treating doctors mentioned above, three doctors were called to give evidence on the Plaintiff's physical condition, impairments, diagnosis and prognosis: Dr. Kwok, an orthopaedic surgeon; Dr. Kagal, a specialist in internal medicine and rheumatology; and Dr. Cameron, an orthopaedic surgeon. The expert evidence is outlined later in these Reasons.

[56] Two doctors were called to give evidence on her psychological condition and impairments, diagnosis and prognosis. Dr. Reznick and Dr. Abrams.

[57] I note that Dr. Kwok saw her in 2005 and 2008, Drs. Reznick and Cameron in 2008, before she started retraining as an accountant and before she suffered the hot oil burn in early 2010.

Plaintiff's Evidence – Fall 2009-Trial

[58] The Plaintiff gave evidence that by the fall of 2009, she had reluctantly concluded that she would never be able to return to her profession because of the lifting, bending and physical activity involved.

[59] In October 2009, because "I had to do something for myself," she started retraining as an accountant. She anticipated that after she retrained, she would be able to find a job and "have a better future." She attended classes at Everest College five days a week from 8a.m. -12 noon. Attending classes caused fatigue. When she returned home, she needed to rest. After she finished studying, she tried to do light cooking and was "mostly able to prepare supper." She had difficulty falling asleep. She said she felt tired; some days she was sleeping in the lectures.

[60] While it was hard for her to concentrate on the accounting course material, she did well and was proud of herself. When she finished, she felt better psychologically because she saw some future for herself. However, she continued to have physical pain.

[61] Just before she graduated from the course in June 2010, she started to send out resumé's seeking work as an accountant.

January 2010

[62] In January 2010, the Plaintiff was cooking with hot oil. She said when she tried to grab a pan with both hands, she was unable to hold the handle. She let the pan go and was severely burned.

[63] Exhibit 1 is a photograph of the scars, taken some months later.

[64] Counsel for the Defendant submitted in her closing to the jury that there was no evidence tying the oil burn to the accident. I disagree. In the medical records, including Dr. Park's notes and other evidence, there is repeated reference to tingling and numbness in her hands. Dr. Kagal said tingling and numbness are hallmarks of fibromyalgia. The Plaintiff's son Pavle, now 14, referred to numbness in her

fingers, seeing her dropping dishes on an ongoing basis. He witnessed the burn incident, when she dropped the pan of boiling oil.

[65] Counsel for the Defendant submitted in her closing that when she went to the hospital in January 2010 seeking treatment for the burn, she didn't mention the February 2005 accident. Since there is nothing in the hospital record reflecting that in 2010 she related the burn to the 2005 accident, the jury should conclude that the Plaintiff did not think the burn was indirectly caused by the accident. It is not surprising to me that at the time the Plaintiff attended at the hospital to seek emergency treatment for the burn, she was more concerned about her immediate problem than with its relationship to the accident.

[66] The burn incident is of significance in my view as it corroborates her complaints of ongoing numbness and tingling in her fingers affecting her ability to work in her original profession and to do heavy cooking. Before the accident she had routinely lifted heavy objects, for instance at Sobeys. It seems highly unlikely that she would have deliberately burned herself with hot oil simply to bolster her accident claim.

[67] Her son Pavle gave evidence at trial that his mother continues to suffer from numbness in her fingers. He said when she tries to do dishes, she often breaks them. A week before the trial, she broke her favourite china dish.

Plaintiff's Evidence re 2010-2011

[68] The Defendant relied upon her admitted padding of her resumé in 2010 in an attempt to find work to call the Plaintiff's credibility into question [and to support its primary argument that the Plaintiff was malingering]. She claimed to have had two jobs she did not have, because she feared employers would not be prepared to overlook a hiatus between the accident and the completion of her retraining in 2010.

[69] Exhibit 2 is a list of job applications the Plaintiff submitted between June 2010 and March 15, 2011, the date when she first started working as a junior accountant at J&A Cleaning Solutions [at first, part-time, 24 hours per week, 8 hours a day on Monday, Tuesday and Thursday.] She gave evidence she was able to tolerate that work because she could rest on Wednesday and the weekends. However, since she did well at the job, she was asked to work full-time. She did so between April 25, 2011 and May 17, 2011.

[70] She gave evidence that she found full-time work much more difficult. While psychologically she felt better, physically she felt worse. As the day progressed, she had more difficulty concentrating. When she went to see Dr. Park on May 17, he advised her to work no more than 25 hours a week. As J&A was unwilling to provide part-time work, that employment ended.

[71] Prasad Subramaniam, her manager at J&A Cleaning Solutions, gave evidence that he found her hard-working, open and honest. He was aware she had health issues 2-3 times involving neck and back

pain. She was always on time. On April 25, 2011 she complained about dizziness and back pain. She said she couldn't sit for 8 hours a day, five days a week. J&A Cleaning did not have office employees working fewer than 8 hours per day.

[72] After she stopped working at J&A Cleaning Solutions, the Plaintiff sought another part-time position. Exhibit 2 lists her job applications between May 17 and September 20, 2011, when she found part-time work as an invoicing clerk at Autobahn Freightlines, five days a week from 9:30 to 2:30.

[73] The Plaintiff gave evidence that she preferred the part-time job at Autobahn Freightlines over a part-time job at J&A Cleaning Solutions because she could tolerate five hours a day, five days a week more easily than 8 hours a day three days per week. Although she felt tired at the end of each five hour day, she could go home, rest, and then do light housekeeping such as dusting and light meal preparation. When she was working, her mood was better but her physical pain was worse.

Evidence of Treatment Providers, May 2011-Trial

Lane, Social Worker: May 2011-Trial

[74] Lane started treating the Plaintiff again on May 25, 2011 and was continuing to see her at the time of the trial.

[75] In May of 2011, the Plaintiff was continuing to have pain, dizziness and sleep disturbance. Working full-time had created problems for her. She was feeling a sense of uselessness and depression, not as profound as before. When she had obtained a part-time job, she had felt that she was finally making money for her family, getting her life back. Lane could see she was still very tired but glad to be working. She was trying to get better. Work provided a distraction from the pain.

[76] Lane had last seen the Plaintiff before the trial on May 9, 2012. Lane opined that the Plaintiff was able to do part-time work. She was still not doing heavy housework. Her household tasks were limited, similar to what they had been in January 2006.

[77] Lane didn't think her memory difficulties were significantly interfering with her daily activity.

[78] Lane said, "There is a genuineness about Biljana. She never evaded questions. She was trying, she thought she could work full-time but it turned out she couldn't. She sought part-time work. She was making an effort."

Dr. Park to the Time of Trial

[79] Dr. Park continued seeing the Plaintiff to the time of the trial. He said she continues to have neck and back pain. His diagnosis of depression was continuing.

[80] Dr. Park's notes mention diagnoses of chronic neck pain and depression.

[81] He gave evidence that although there were periods when she was suffering from low iron or inadequate thyroid treatment, those were corrected and should not have been a continuing factor.

Expert Evidence on Her Physical Condition

Dr. Kwok – June 2005

[82] Dr. Kwok, orthopaedic surgeon, saw the Plaintiff on June 14, 2005 and performed an orthopaedic examination. When he asked her to walk he noted she had an unsteady gait. She had tenderness in the middle and both sides of her neck and in the left and right supra scapular areas.

[83] He said in his view, tenderness and range of motion were "objective" if they were reproducible. It would have been difficult for her to repeatedly "fake" pain in exactly the same way. He noted she had a reversal of normal lumbar rhythm, an indication of back pain.

[84] Significantly in my view [especially given the evidence of Dr. Cameron that muscle spasm cannot be faked on examination], on examination Dr. Kwok found muscle spasm in the upper and lower back. His evidence dealing with muscle spasm was as follows.

A. She has muscle spasm.

Q. How does that work?

A: When you palpate, that means when I feel the muscles, the muscle is very supple but when there's muscle spasm the muscles become very tight.

Q: Did you notice that; did you do that test with her?

A: Yes.

Q: You said you did notice muscle spasm?

A: Yes.

Q. You mentioned where, though?

A: She has back muscle spasm.

Q. What part of her back?

A. In the low back and upper back.

[85] He opined she had injured her shoulders, chest wall, spine, neck and back in the accident, causing disability and inability to lift, bend, squat and kneel. He concluded that at that time she wasn't able to return to work or to do heavy housekeeping and caregiver tasks.

[86] Dr. Kwok said her history, examination and the tests he performed all fit together. He recommended additional treatments.

[87] Dr. Kwok did not provide a prognosis in 2005.

Dr. Kagal: Examinations - October 30, 2008 & July 11, 2011

[88] Dr. Kagal, a rheumatologist called by counsel for the Plaintiff, gave evidence that there is an overlap between the work of rheumatologists and orthopaedic surgeons. Orthopaedic surgeons deal generally with surgical aspects and generally don't diagnose or treat non-surgical issues such as chronic

pain syndrome. Orthopaedic surgeons generally refer non-surgical issues such as chronic pain syndrome to rheumatologists.

[89] Dr. Kagal saw the Plaintiff on October 30, 2008. He saw her again on July 11, 2011.

[90] He said he tried to rule out other possible causes for her pain, including arthritis. He eventually concluded she was suffering from fibromyalgia, a recognized chronic pain syndrome, a common condition occurring in between 2-12% of the population. Fibromyalgia patients have physical symptoms including fatigue, loss of concentration, discomfort and a sensation of numbness and tingling in the hands and feet. In his opinion, she met the diagnostic criteria. She had chronic headaches, difficulties with concentration which by 2011 were interfering with her ability to work as a bookkeeper. The pain and all the other symptoms – fatigue, headache, reduced concentration - interfered with her ability to work.

[91] Fibromyalgia is a condition that has been validated by the American College of Rheumatology, which in 1990 set out criteria based on a detailed physical examination. A patient must have 11 out of 18 fibromyalgia tender points for the diagnosis to be made. Two years ago, the College included a cumulative pain score and incorporated a patient's assessment of their somatic symptoms as well. Criteria such as fatigue, feeling unrefreshed in the morning and other somatic symptoms were incorporated into the diagnosis. Just saying "pain all over" doesn't make it fibromyalgia.

[92] Of Dr. Cameron's evidence [outlined later in these Reasons] that fibromyalgia is not real, is not a disease, is too easy to "game," Dr. Kagal said although fibromyalgia is not a disease, it is a syndrome, a collection of symptoms for which the exact cause is unknown. Rheumatologists are trained to know the exact location of the fibromyalgia tender points and can tell if a patient is credible by the use of control points. His evidence includes the following:

Q. How can you tell ... if they're telling the truth or not?

A: First part is the control point. Someone ... will complain of pain in other areas that are not tender points either has another process going on or, you know, are not telling the truth. It's also the degree how deeply [you] palpate as well when you're diagnosing fibromyalgia. If ... we're going to palpate someone's muscle let's say I'm going for example this is a tender point right here, there is the elbow two centimeters below the elbow, if I press hard on anyone they're going to feel pain. You're supposed to palpate very lightly, very very lightly, just enough to cause your fingernail to blanch. All right. So it's very light, all right. That's another way of controlling the issue. Another issue is how patients react during examination. There are other ways of knowing or trying to pinpoint ... whether the pain is real or not. A lot of it is subjective to be very honest but there's certain behaviours of mimickers such as overexpression, being hypervigilant if a patient, you know, cries of pain even before you touch them; there are also other validated physical examination maneuvers to distinguish between true pain and what's called "malingering[ing]." And what would those be? One of them is, for example, the axial node sign. You palpate someone's head and say, okay, am I causing pain there. If they say no, great. Palpate the head over here and just feel pain anywhere do you feel in your lower back. If someone's not a mimicker or someone's not malingering, they'll say, no, they don't feel pain.

Q.: Out of the 16 or 18 tender points how reliable are those tests, the pin prick and the 18 tender points. Can someone Google that and is it possible for someone to fake that, those tests?

A: Yes ... I'll give you an example. If you look at Google, they're going to put a big dot on the elbow, you know, if you press the elbow in fibromyalgia, it's going to be normal. So someone complained of pain in the elbow you know that -- you know what that's something that's not true. Actually the tender point is actually two centimeters below. So I don't think you're going to find that on Google, you know, and also control points are not found on Google either.

[93] As to his examination of the Plaintiff, his evidence was as follows:

Q. What was your impression in terms of her presentation on that day?

A. First thing I want -- I want to premise is she's not a regular patient of mine. That's the first thing. But I do recall reading from my note or my recollection that she was co-operative. Her affect was flat. She did not -- she did not tend to exaggerate her symptoms in any way. If anything, it was the exact opposite. She seemed quite withdrawn at the time.

[94] When Dr. Kagal was asked about Dr. Cameron's "fake" test results, he said he was surprised that Dr. Cameron found she failed the axial-loading test. He said he performs an axial loading test "automatically and generally." He thought it did not indicate the Plaintiff was faking. However, since he did not record it in his notes, he could not say he did that test "for sure."

[95] With respect to Dr. Cameron's conduct of the axial loading test, Dr. Kagal said he "would want to know how much force Dr. Cameron put on her head."

[96] On Dr. Cameron's evidence that fibromyalgia is not caused by trauma, Dr. Kagal gave the following evidence:

A. Fibromyalgia can be caused by non-traumatic events. But what we do know is there are studies that have shown a strong link of trauma to fibromyalgia. In fact, recently there's a study that was published by the journal arthritis rheumatism, which is the leading research journal of the American College of Rheumatology, where they actually followed patients prospectively.

Why these important because many times when you look at medical literature there is speculation of risk factors in fibromyalgia. They're all based on retrospective studies, in other words, looking back. So in this study they actually look to patients who did not have chronic pain syndrome or fibromyalgia and followed them over four years. Over those four years 12 per cent of those patients are people or clients developed chronic pain syndrome or fibromyalgia and out of those 60 per cent were able to identify specific trauma. And the interesting part of that is that when you look at all the various types of traumas that one can -- specific traumas one can have, including hospitalization, stress at work, fracture, even medical illness, the only one that was statistically significant was motor vehicle accident. So there's still lots to learn about fibromyalgia. We don't know what it's about, the motor vehicle accidents or even trauma in general that causes fibromyalgia, but we do know there is a strong association.

[97] Dr. Kagal was "very confident of the diagnosis" [of fibromyalgia.] Her prognosis is poor. She will probably continue to have chronic widespread pain indefinitely, with no reversibility. It is extremely unlikely that she will be pain-free in the future. Her chances of remaining in the workforce for the foreseeable future as poor. There is over a 90% probability that she will retire before age 60.

Dr. Cameron: Examination - September 26, 2008

[98] Counsel for the Defendant called Dr. Cameron, an orthopaedic surgeon, to give evidence about the Plaintiff's physical condition, diagnosis and prognosis. At the request of the defence, Dr. Cameron interviewed the Plaintiff and conducted an orthopaedic examination of the Plaintiff for about half an hour on September 26, 2008.

[99] Dr. Cameron performs surgery on 10-12 joints a week and said he "operates in chronic pain all the time." He said if treating painful and damaged joints is included in chronic pain, then "we see lots of chronic pain." However, in cross-examination, he conceded that in the hundreds of times he has given evidence at trial, he has never given expert evidence on chronic pain "if you're assuming that it's a psychiatric condition." He was asked in cross-examination:

Q. So in terms of chronic pain that has a psychological element how many times have you been admitted as an expert to the court?

A: I couldn't comment on that.

Q: Why is that?

A. Because I'm not a psychiatrist.

...

Q. Clearly, you've never diagnosed someone with fibromyalgia?

A: No, I see a whole bunch of painful guys.

Q: I'm sorry, doctor?

A: There's a whole bunch of people with soft tissue complaints, diffuse soft tissue complaints. It's a useful name, even though it doesn't mean anything.

[100] In his evidence, Dr. Cameron made repeated reference to his "fake tests" on the Plaintiff, which caused him to conclude she was exaggerating. For example, he said he did a fake test consisting of bending her knee and asking if it was painful. She complained of back pain. [Since the pressure was off the spinal nerves, it should not have produced back pain.] Because she complained of pain, he found "lots of evidence" of symptom exaggeration.

[101] He said she kept saying she was dizzy. If she was dizzy with vertigo, her eyes should have moved from side to side and flickered. Hers did not. [In cross-examination, he admitted that one does not get eye flickering with wooziness.]

[102] I have already referred to the evidence of Dr. Kwok, another orthopaedic surgeon, who said he found muscle spasms on his examination of the Plaintiff in 2005: Of muscle spasm, Dr. Cameron said:

Question: You said there was no muscle spasm?

Answer: Yes.

Question: Is that significant?

Answer: Yes. You don't often see muscle spasm but when you see it it's not something that could be faked.

[103] I note Dr. Cameron did not refer in chief to Dr. Kwok's observation of muscle spasm, even though he said he reviewed Dr. Kwok's reporting letter dated June 14, 2005, which mentioned

"evidence of muscle spasm" at p. 10. In cross-examination he questioned Dr. Kwok's methodology, saying Dr. Kwok failed to do a passive examination of the Plaintiff.

[104] On fibromyalgia, Dr. Cameron's evidence in cross-examination contains the following:

Q: Thank you. You're not a psychiatrist; you're not a rheumatologist?

A. No.

Q: If I propose to you a rheumatologist is an expert in fibromyalgia how would you comment?

A. I would not accept that.

Q. Why is that?

A. Fibromyalgia isn't a disease. And so is it fair to say you just don't believe in fibromyalgia? Yes.

...

Q: You don't actually diagnose anybody that you have seen with that?

A: Generalized painful people I will tell them they have fibromyalgia.

Q: Would you actually give that diagnosis, even if you don't believe in it?

A: Yes.

Q: How is that?

A. Well, because, you know, what else are you going to call somebody who aches from top to toe.

Q: What would you do then; how would you treat someone with fibromyalgia?

A: Send him to see a physiotherapist for a structured exercise program.

Q. You would not provide any treatment yourself?

A. There is no treatment.

...

A. Fibromyalgia is one of those sort of pseudo diseases. It's not new. My mother knew about it. It's never been demonstrated being a disease. It's never being demonstrated pathologically. There are no imaging studies. People disagree what the physical findings are. No one can decide what the cause of it is. It's not supposed to have any cause.

Q: Are you any aware of any tests to diagnose this?

A: Yes, there is a poke test. You poke people here and there. If they say ouch, they got a disease.

Q: Dr. Cameron, fibromyalgia, is it a disease or condition that certain specialties qualify to give evidence on?

A. Anybody can give evidence on fibromyalgia.

Q: Why do you say that?

A. Because it's a widespread condition. It's widely greatly diagnosed, but no one has shown it's actually a disease; that it actually exists.

Q. Is it something that an orthopaedic surgeon would treat, fibromyalgia?

A: There's nothing. With all due respect, Your Honour, there's nothing to treat. The standard treatment for fibromyalgia is encouraging them to exercise and that's it. There's no drugs. There's no special nothing. It's just exercise.

The Court:: Is that something that an orthopaedic surgeon would receive training in?

A. Oh, yes.

Q. Dr. you mentioned training in all musculoskeletal areas.

A. Well, as a resident you're in a clinic with a consultant and also everything that comes through the door has to be examined. Everything that comes through the door, that means all aches and pains fibromyalgia or otherwise. All aches and pains, soft tissue pains have got to be examined. So you have to be able to know if this is a significant condition which needs treatment or an insignificant condition. You have to be able to make the distinction.

The Court: Do you know that at the very beginning?

A: You get to know it pretty soon, quickly, Your Honour.

The Court: How do you get to know whether it should be treated or not?

A: It's experience and being taught by the consultant.

Q. Who is the consultant?

A. Well, when I'm teaching medical students for example I'm the consultant.

Q.: So this is something you would have learned about when you were training?

A. Yes.

Q: You are familiar with the test that some people might use to diagnose it?

A. Yes.

Q. Can you describe for me what the test involves.

A. You're supposed to poke at them in 18 separate areas and if they say they have pain there then that's the so-called diagnosis. The only problem is that you must do controlled studies, control area testing, and it's way too easy to game it.

Q. To what?

A. To game the situation. The patient can control the situation.

Q. Would that be subjective or objective?

A. Very, very subjective.

Q> Is that a test you use regularly?

A. No.

Q. Why not?

A. It's meaningless.

Q. And Dr. Kagal mentioned in his evidence about noticing a person's reaction, grimacing was the word he used. Do you consider that when you're examining a patient?

A: Oh, yes, you consider it. It's a factor.

Q: And what would you conclude if somebody was grimacing that they had pain or they didn't have pain?

A: again, it's too easily gamed. I mean, in the presence of symptom exaggeration, wincing or shouting out in pain doesn't mean anything.

[105] Dr. Cameron concluded she had sustained only soft tissue injuries in the accident. There was no evidence of any organic pathology. She had not sustained a serious injury of a physical nature. Her recovery should have occurred long since.

[106] Dr. Cameron could find no organic physical explanation for her complaints of pain. He predicted no ongoing or future physical problems as a result of the accident.

[107] Dr. Cameron conceded that he was not an expert on psychological matters, that accidents can cause both physical and psychiatric sequelae, that pain experienced over time can be disabling and can cause psychological sequelae including depression, and that symptom magnification doesn't necessarily mean that a patient is not having pain. He said he is beginning to think that even though pain cannot be measured, it can still be real. He agreed that perceptions of pain can be influenced by anxiety and depression.

Expert Evidence re Psychological and Cognitive Impairments

[108] The four witnesses who gave evidence on the psychiatric/psychological effects of the accident were Dr. Abrams, Dr. Steiner, Dr. Reznick and Dr. Park.

[109] Apart from the family physicians/treating doctors, two psychiatrists Dr. Abrams and Dr. Reznick gave expert evidence relevant to whether the Plaintiff suffers from a permanent serious impairment of an important psychological function.

Dr. Reznick: October 1, 2008

[110] Dr. Reznick's evidence was extremely important to the defence because it was the only evidence before this Court to the effect that she is not suffering from any clinical psychiatric condition and because it was contrary to the evidence of the Plaintiff, her family, Lois Lane, Dr. Steiner, Dr. Park and Dr. Abrams.

[111] Dr. Reznick opined that the Plaintiff was malingering when he saw her in 2008 based on psychological [validity] tests he administered personally, characteristics he said she exhibited during the interview, and his reviews of reports of others who were not called to give evidence. [I did not allow Dr. Sapienza to give evidence in part because Dr. Sapienza's raw test data was unavailable.]

[112] Dr. Abrams, the other psychiatrist who gave evidence, said that generally psychiatrists do not do psychological testing. Psychologists do. In determining whether a patient is malingering, psychiatrists do apply the DSM IV. Unlike Dr. Abrams, Dr. Reznick did not refer to the DSM IV test for malingering nor did he apply it.

[113] Contrary to the evidence of Dr. Abrams and Dr. Kagal, Dr. Reznick suggested that psychiatrists frequently do validity tests and that they should have done validity testing on the Plaintiff.

[114] Although the test results he said he obtained were abnormal, Dr. Reznick did not attribute her poor test results to difficulties in concentration or English language difficulties. He did not ask her whether she had such difficulties.

[115] Ms Cantor suggested in cross-examination that Dr. Reznick may have overstated the poorness of her results on the Folstein test to support his conclusion that she was exaggerating her symptoms.

[116] Dr. Reznick referred specifically to taking her pulse rate when she was talking about the accident. He emphasized that that was an "objective" test. He used the fact that her pulse rate did not markedly increase in concluding she was exaggerating. In cross-examination he acknowledged that the pulse rate test was a test of his own design, not recognized as valid in the research. When he was unable to provide evidence that others considered it a legitimate test, he said "I don't know why others don't use this as a test. To me they aren't being smart enough."

[117] Dr. Reznick referred in evidence to certain questions contained in a psychological malingering test he did not personally administer. Since he did not have the raw test data of the psychologist who did, he could not know how the Plaintiff responded or would have responded to the questions he quoted.

[118] He did not explain how or why a "malingerer" would retrain so she could find work despite an ongoing lawsuit. He said "more credit to her that she's managed to get unstuck from something that she got caught in and has then gone on to be as functional as she can be.. I haven't seen her subsequently so I can't comment on ... whether she has stopped exaggerating her symptoms." In other words, although he opined that she was malingering in 2008, he could not comment on whether she was malingering in 2012.

[119] Dr. Reznick saw the Plaintiff for about an hour and a half in 2008. He said that he didn't see objective evidence that she was suffering from severe pain. In effect, he said she appeared to be enjoying his interview. He made a note every time she smiled (5-10x) during the interview. Despite acknowledging that she knew he was conducting a psychiatric assessment on behalf of the Defendant, and that he wrote elsewhere in his report that she was guarded and it was hard to establish rapport, he maintained that she was genuinely laughing at his jokes about being the baby in the family and meeting her husband on the street.

[120] He looked for major depression, adjustment disorders, post-traumatic stress disorder and pain disorder, and concluded that she didn't fulfill the diagnostic criteria for any of them.

[121] Under the DSM IV, a diagnosis of depression requires fulfilment of at least five of the DSM IV criteria.

[122] Dr. Reznick said when he examined her, several were present, including sleep disturbance and fatigue, reduced appetite, weight gain. He didn't feel she had inappropriate feelings of guilt because he felt her guilt feelings were appropriate.

[123] He noted she had occasional thoughts that others would be better off if she were dead but did not take that to be an indicator of depression.

[124] He did not note a diminished ability to think or concentrate.

[125] In reaching his conclusion she was not clinically depressed, he mentioned that she has the capacity to enjoy things like going out, socializing, time with her husband, playing with her children. He did not find a depressed mood for most of the day. He discounted the evidence of her son Pavle, describing his mother as "really depressed," "like someone drained all the energy out of her." He said she did not have a loss of joy.

[126] He found no foreshortened sense of future, despite the fact that he quoted her as saying, "I dream I have a car accident and I'm dead." He said "a dream is a dream." He had found no flashbacks "when she was awake."

[127] He did not diagnose a pain disorder because he didn't think she was expressing an emotional event as physical symptoms. He said she may have had some pain but in his view it did not reach a threshold level for a psychiatric diagnosis.

[128] He did not diagnose an adjustment disorder, a disturbance of mood or anxiety more than would be expected in the circumstances.

[129] He did not diagnose a post-traumatic stress disorder.

[130] Dr. Reznick appears to have erred in using information taken from Exhibit 29 that she was able to enjoy spending time with her husband, going for walks, paying with her children and socializing together. After reviewing Exhibit 29, I conclude that in the interview, she made those comments in relation to her pre-accident state. The note appears on p. 3, after her reference to meeting her husband, before her reasons for leaving Serbia, before mention of the accident on page 4, and before "current" on pages 5-7. I reject Dr. Reznick's evidence in cross-examination that his note about "enjoying spending time with her husband, socializing" related to her post-accident state because he used the word "enjoy" in the present tense.

[131] Since he concluded she was not suffering from any psychiatric disorder, he inferred that she had no disability and no psychiatric problem. She was not suffering from any psychiatric impairment and was not disabled from pursuing any career of her choice.

[132] It is worth noting that Dr. Reznick saw her once in 2008, before the hot oil injury, before her re-training, before she attempted to work fulltime after that retraining, before Dr. Park's letter dated May 17, 2011 to the effect that she should not work more than 25 hours a week.

[133] He did do a subsequent paper review.

[134] In short, I reject Dr. Reznick's conclusions on depression that were based on an apparently incorrect assumption, unsupported by the other evidence, including his own notes, that after the accident, she enjoyed socializing, spending time with her husband, etc.

[135] His description of his observations of the Plaintiff was at odds with most of the rest of the evidence. The woman described by Dr. Reznick at his interview was not the woman described by Mr. Beader, Pavle Beader, Dr. Park, Dr. Abrams, Lois Lane, Dr. Steiner, or any of her other employers or contacts after the accident. His observations as to her demeanour during the interview were at odds with her evidence, the observations of her family and of every other doctor who gave evidence.

Dr. Abrams: January 18, 2010

[136] Counsel for the Plaintiff called Dr. Abrams, like Dr. Reznick, a psychiatrist. Dr. Abrams saw the Plaintiff for 1-1/2-2 hours on January 18, 2010.

[137] Dr. Abrams disagreed with Dr. Reznick's conclusions that the Plaintiff was malingering, in part because he was using a methodology/psychological testing not usually administered by psychiatrists. Dr. Abrams said that she used the DSM IV criteria of malingering, as other psychiatrists do. Applying those criteria, she did not find the Plaintiff to be malingering. The criteria were as follows: 1. Medico-legal context; 2. Discrepancy between the patient's complaint and objective findings; 3. Evidence of a discrepancy between what was recorded and the findings; 4. Lack of cooperation with medical treatment providers or prescribed medical treatments; 5. Anti-social personality disorder. Although there was an ongoing lawsuit, Dr. Abrams found no discrepancy between the Plaintiff's complaints and presentation and her objective findings. The history she gave was consistent. Her cognitive testing was in keeping with what she reported and what Dr. Abrams observed. She had seen many different psychiatrists, psychologists, had had psychological treatment, various psycho-therapies and various biological treatments, multiple anti-depressants, etc. She had been cooperative with her treatment providers and compliant with medical treatments. The Plaintiff didn't have the personality of someone who would take advantage. She denied many symptoms/was not over-endorsing them. She appeared to be making a strong effort. For example, when asked to do so, she was able to recite the months backward.

[138] Using the DSM IV, she made a clinical diagnosis, among others, of depression. A patient must at a minimum meet five of the various criteria for depression within a two-week period. Dr. Abrams opined that the Plaintiff fulfilled seven: 1. Depressed mood, nearly every day, all day; 2. Diminished interest in pleasure in activities; 3. Changes in weight or appetite/weight gain; 4. Insomnia; 5. Loss of energy; 6. Feelings of worthlessness; and 7. Diminished ability to concentrate.

[139] In her opinion, the Plaintiff presented very genuinely and in keeping with a person who was depressed. Her affect was dysphoric, blunted, sad: she was not laughing or smiling. She said emotional factors were contributing to the Plaintiff's continuing perception of pain. When a person is depressed, pain worsens.

Present Condition

[140] Dr. Abrams opined that at present, the Plaintiff's major depression is causing significant impairment in her social and occupational functions. Her psychological impairment is affecting her activities of daily life including her ability to work, to care for her home and to be successful.

[141] Her problems will likely continue. She will likely need prolonged therapy over "most likely years" including medication management "probably long term."

Dr. Steiner: August 2005-January 2006 and May 2011-Present

[142] Dr. Steiner disagreed with Dr. Reznick's conclusions. Dr. Steiner concluded that she had developed a chronic pain disorder with severe restrictions in carrying on the activities of her life. He said he and Ms Lane had interacted with the Plaintiff over many months. He criticized Dr. Reznick's reliance on Dr. Marton's conclusions without having had access to Dr. Marton's raw test data. He said he and Lane had a more complete impression of the Plaintiff's situation than Dr. Reznick, whose conclusions were based on one short interview and examination.

[143] He noted that Dr. Reznick was relying on his own psychological testing. Generally psychiatrists do not do psychological testing. Dr. Steiner, a psychologist who generally does do psychological testing, said Dr. Reznick's results were highly unusual. He questioned Dr. Reznick's conclusions based on the psychological tests Dr. Reznick did.

[144] Had Dr. Steiner obtained the results Dr. Reznick purported to have obtained on the psychological testing he did, Dr. Steiner would have questioned those results. Dr. Steiner has conducted the Wray memory test hundreds of times and has never had the result Dr. Reznick said he obtained: a recall of only two items. Dr. Steiner wondered whether the Plaintiff understood what she was being asked to do on the test.

[145] Dr. Steiner opined that Dr. Reznick failed to incorporate the Plaintiff's history and determination to return to work despite her pain. The conclusion that her capacity to work was not affected by the accident was illogical, given that at the time of the accident she was working two jobs; she was "hell-bent on going back to work;" she attempted to re-train; she pushed herself even when she was in pain; she was distressed when she could not meet the demands of a full-time accounting job.

[146] Dr. Steiner said that the Plaintiff was a credible, sincere, genuine person. In his opinion, it was highly unlikely she was malingering. Had they thought she was misrepresenting information, he and Lois Lane would have stopped treating her immediately.

Evidence re Continuing Disability and Impairments at the Time of Trial

[147] Dr. Kagal's evidence contains the following:

A: Again, I think that what – I think that her chances of remaining in the workforce would – for the foreseeable future in the long term future is very poor.

Q: So could you give us any kind of probability. What was the probability that she would have to retire before age 65 for example?

A: Very high probability.

[148] The Plaintiff and Mr. Beader gave evidence that the accident changed their dreams and expectations.

[149] The Plaintiff gave evidence that at the time of trial, she was continuing to have concentration difficulties that usually worsened as the day progressed, headaches, neck pain, back pain and shoulder pain, dizziness and numbness in her fingers probably 85% of the time in the fingers of both hands. In a 7 day week, she feels normal 2-3 days, and sad the rest of the time. She is more depressed when she is at home for example, when she sees her older son Pavle doing work she thinks she should be doing. He helps her younger son and takes him to and from school. He also does laundry and cleaning. She feels guilty because her husband must work two jobs. [He now is working 40 hours per week, 5 days a week, plus delivering pizza for an additional 24 hours per week.] Noise bothers her. She doesn't watch TV. She has less patience with her husband and children. She is still afraid to ride in a car. She prefers public transit than in a car. Her concentration is poor.

[150] When she goes outside and sees people she knows, when she smiles and laughs, it is "just in her mouth, not in her heart." She doesn't want to see "sorry" in their eyes. She tries to find excuses not to see other people.

[151] Mr. Beader gave evidence that she is a different person today. Although she is really trying hard, she is not able to work fulltime. Before the accident she was joyful and proud; now she is constantly complaining, feeling dizziness, having nightmares, struggling and not being able to do what she wants to do. When asked in cross-examination whether any money recovered in this lawsuit would help the family financially, he said, "Certainly money would help now. But what she lost – the health and in general – is something that money can't buy. Money cannot repay."

FINDINGS OF FACT ON THRESHOLD MOTION

[152] I accept the evidence of Dr. Kwok that he found muscle spasm and tenderness that was reproducible in 2005.

[153] I accept Dr. Cameron's evidence that muscle spasm cannot be faked.

[154] Since the accident and continuing to the present, I find the Plaintiff has had widespread continuing pain related to the accident. I accept the Plaintiff's evidence that she continues to suffer from numbness and tingling in her arms, hands and neck, headaches, back and shoulder pain. She has nightmares and difficulty sleeping.

[155] I accept the evidence of Dr. Park that she has fatigue from working with pain. Daily headaches and difficulty sleeping and nightmares contribute to her daily fatigue.

[156] I accept Dr. Park's evidence about the consistency of complaints over many years.

[157] I find that in 2011, Dr. Park opined the Plaintiff should not work more than 25 hours per week.

[158] Starting in August of 2005, Dr. Park was sufficiently concerned about her psychological state that he prescribed anti-depressants and referred her to a psychiatrist, who continued and significantly

increased the dosage of the anti-depressant medication that Dr. Park had initially prescribed. In 2010 Dr. Abrams, a psychiatrist, diagnosed major depression. Drs. Park, Abrams and Steiner all found her presentation to be consistent with depression.

[159] I accept Dr. Abrams' evidence that her affect was dysphoric, sad, not laughing or smiling and that she could not get pleasure or joy from pretty well everything, activities she normally would have.

[160] I find the Plaintiff continues to suffer from depression, which relates in part to her perceived pain, her inability to work fulltime, and her guilt about not being able to carry her own weight [her husband having to work two jobs and her son Pavle having to do work she would liked to have done herself.]

[161] I accept Dr. Tamari's evidence that the Plaintiff had no significant pre-accident conditions and Dr. Tamari's and Dr. Park's evidence that by the time of the accident and thereafter, her thyroid condition and iron deficiency had responded well to medication.

[162] In respect of her psychological impairment and its level of seriousness and likely duration, I accept the evidence that at the time of trial she was suffering from chronic pain, continuing major depression and perception of pain, and a continuing inability to work on a fulltime basis. She could not find joy in life, relate happily to other people, or sleep well at night. She was having recurring nightmares at least two times a week.

[163] In determining whether or not the Plaintiff suffered a serious, permanent impairment of an important psychological function, I have not accepted the opinion of Dr. Reznick that she was malingering and that she was not suffering from any clinically diagnosable psychiatric condition.

[164] Over the material years, her affect was consistently flat. She was sad. She was struggling. She wanted to get better. She wanted to work. She was frustrated and depressed. When she retrained and tried to work full time in a less physically demanding role, she could not. That, too, was depressing.

[165] Her psychological impairments affect her ability to work fulltime, do heavier housework and otherwise function normally. While she can work part-time, it is unlikely that she will ever be able to work full-time.

[166] I find, based on the evidence of Doctors Park, Kagal and Abrams, that it is unlikely that her pain will ever resolve. With respect to prognosis, in part because her condition is chronic, I accept the evidence of Dr. Kagal and Dr. Abrams that she needs further treatment.

THE LAW

Analysis

The Issue in this Case – Application of the Law

[167] The issue is whether the injuries sustained by the Plaintiff in the accident have resulted in a permanent, serious impairment of an important physical, mental or psychological function within the meaning of s. 265.5(5) of the *Insurance Act*.

Does Her Present Condition Relate to the Accident?

[168] On the evidence here, I am of the view that the Plaintiff has proven on a balance of probabilities that her impairments relate to the accident.

[169] Before the accident, she was working two jobs. Her thyroid condition was controlled by medication. Her low iron was controlled by medication and in any event would not have caused the kind of pain she is continuing to experience.

[170] I accept the evidence of Dr. Park that her thyroid condition and iron deficiency were controlled by medication and could not or would not account for the widespread pain and the weakness from which she now suffers.

The First Question: Are the Injuries Permanent?

[171] I have borne in mind that the purpose of the *Act* was to significantly limit the right of victims of motor vehicle accidents to maintain tort actions against tortfeasors for the reasons already outlined. I am satisfied, based on the evidence as a whole, including the evidence of Dr. Kagal and Dr. Park, outlined earlier, that the Plaintiff's injuries are permanent. Similarly, I am satisfied that her impairments are permanent.

Application of the *Meyer v. Bright* Test

[172] In *Meyer v. Bright*, [1993] O.J. No. 2446 (C.A.), the Court of Appeal mentioned that some injuries which are physical in nature can be diagnosed objectively. Some can only be diagnosed based upon the patient's subjective complaints. Others are diagnosed on the basis of both objective observations and the patient's subjective complaints. In this case, I have accepted the evidence of Dr. Kwok that he observed muscle spasms in 2005. Dr. Kwok opined that her physical presentation of tenderness and reproducible pain was not being faked. Dr. Cameron conceded muscle spasms are objective symptoms that cannot be faked.

[173] I accept the evidence of Mr. and Mrs Beader, Ms Lane and Dr. Park that she has suffered pain since shortly after the accident.

[174] Her pain, numbness, fatigue, tingling and headaches are continuing to this day and on the evidence are likely to continue into the future. I accept the diagnosis of chronic pain and depression.

[175] In *Page v. Primeau*, [2005] O.J. No. 4693 (S.C.J.), H.G.H. Sedgwick J. wrote at paragraph 30 as follows:

[30] ...Chronic pain arising from injury sustained in a motor vehicle accident and resulting in limitation of function which is unlikely to improve for the indefinite future meets the requirement of “permanent” ...

[176] Her psychological depression resulting from the pain, fatigue and her inability to work full-time, has contributed to her ongoing disabilities.

[177] I have found her combination of mental, psychological and physical impairments, on a balance of probabilities will be permanent.

The Second Question - Important

[178] This question requires a determination of whether the bodily functions and mental and psychological functions that have been impaired are important bodily, mental or psychological functions.

[179] In *Meyer*, the Court of Appeal directed that this criterion should be interpreted and applied without resort to so-called objective or subjective tests. The Court of Appeal directed that the bodily function [or mental or psychological function] referred to must be one which is important to the particular injured person. While there are functions that are obviously important to everyone, the application of the legislation requires the Court to consider whether bodily functions are important to some people but not others. The legislation was aimed at bodily functions that play a major role in the health and general well-being of the injured plaintiff. It was an issue of fact to be determined on the evidence in each case.

[180] I have accepted the evidence that although she made a concerted effort to work at Sobeys despite her pain, and tried over a lengthy period to recover the physical capacity that would enable her to perform the physical requirements of the profession for which she was trained, she was not able to do so. Therefore, in 2009, she resolved to retrain for a less physically demanding occupation.

[181] She undertook an accounting course in the hope that she would be able to work full-time. After successfully completing the accounting retraining, she attempted to work full-time in that capacity. On May 17, 2011, on the recommendation of Dr. Park that she should not work more than 25 hours a week, she left the accounting job given that her employer required an accountant to work on a full-time, 40 hour per week, basis.

[182] She then sought other part-time work.

[183] She followed recommended treatments for physiotherapy, drug therapy, etc.

[184] I have found that she is also continuing to suffer from depression. She does not experience joy in everyday living, as she did before the accident. In light of all the above, she suffers from impairment of important bodily and psychological functions.

The Third Question: Is the Permanent Impairment Serious

[185] The answer to this question requires a determination of whether the permanent impairment of the important bodily, mental and/or psychological function is serious. Serious relates to impairment and not to injury. A serious impairment is one that causes substantial interference of the injured person to perform his or her daily activities, or to continue his or her regular employment.

[186] Here, I have accepted the evidence that the Plaintiff wants to get better, wants to work full-time, has been compliant, and that the degree of impairment goes well beyond frustrating and unpleasant.

[187] In this case, having considered the degree of impairment through the *Meyer v Bright* lens, I would classify her impairments as serious, her depression as major, her weakness serious enough to cause a serious burn. Her limitations of movement and tenderness, pain and headaches continue.

[188] Cases such as *Dinham v. Brejkaln*, [2005] O.J. No. 5409 (S.C.J.), where the plaintiff failed to prove that headaches were caused by the accident; and *Bridgewater v. James*, [2004] O.J. No. 5282 (S.C.J.), where H.J. Wilton-Siegel J. concluded that the continuing impairments were not caused by the 1998 accident, are not helpful.

[189] In *Page*, Sedgwick J. wrote at paragraph 33: "As to the impact of these impairments and functional limitations ... there is a sharp contrast between the testimony of the plaintiff in chief and during cross examination." He simply did not believe her evidence with respect to the nature and extent of her impairments of function. He found that she did not follow medical advice.

[190] In *Frankfurter v. Gibbons*, [2004] O.J. No. 4969 (Div. Ct.), the Divisional Court held that an impairment was not serious where it was found that the plaintiff left her job not because of her injuries, but to get married. Although she complained of pain, she was still able to do ballroom dancing and housekeeping. In that case, the Court found that her impairments did not go beyond the tolerable to the serious.

[191] In *Dennie v. Hamilton*, [2006] CanLII 35618 (S.C.J.), O.J. No. 4227, Whalen J. wrote at paragraph 38:

[38] I agree with the defence that substantial interference with daily activities and near inability to continue work are sure markers of seriousness. However, the test is more nuanced where the person has continued to work and tried to do other activities of daily life, as discussed in the case of *Hartwick v. Simser*, [2004] O.J. No. 4315 (at paragraphs 89 and 90) where Roccamo J. held:

‘89 It is now clearly established that one who can carry on daily activities, but is subject to permanent symptoms including sleep disorder, severe pain, headaches, having a significant

effect on the enjoyment of life, will demonstrate symptoms constituting serious impairment. Ongoing and debilitating pain, even in the absence of objective findings by medical experts, will constitute serious impairment: see *Chrappa v. Ohm*, [1996] O.J. No. 1663 (S.C.J.); *Skinner v. Goulet*, [1999] O.J. No. 3209 (S.C.J.); *May v. Casola*, *supra*.

90. A serious impairment is determined by reference to the condition and situation in the life of a particular injured person. What is serious to some will not be serious to others. What is serious must be resolved on a case by case basis. However, a serious impairment is generally one that substantially interferes with a person's ability to perform his usual activities of daily living or continue employment. Even where full-time employment is resumed, albeit in a reduced capacity or in an altogether new area from that performed by a claimant pre-accident, this will constitute a 'serious' impairment: see *Lento v. Castaldo*, [1993] O.J. No. 2446 (C.A.)

[192] The circumstances here are very different from those in the cases provided to me, where the impairments were found not to be serious, including *Perricone v. Baldassarra*, [1994] O.J. No. 2199 (S.C.J.); *Aiello v. Dalton*, [1995] O.J. No. 3652 (S.C.J.); *Houang v. Garcia*, [2002] O.J. No. 2805 (S.C.J.); *Xiao v. Gilkes*, [2009] O.J. No. 735; *Smith v. Sabzali*, 2005 O.J. No. 214, 2005 CanLII – Court File No. 64183/02 (S.C.J.)

DISPOSITION

[193] The threshold motion is therefore denied.

M.A. SANDERSON J.

Released: